

26. PRACTICE : THE LUNGS

DURATION : 05:35

STUDY SHEET

COURSE SUMMARY

This video focuses on the biodynamic approach to the lungs, emphasising how the rocking motion of the lungs and the search for the embryonic fulcrum point are essential for restoring their primary function. Participants will learn to observe the dynamics between the lungs, pleura, and surrounding tissue, including how emotions, such as sadness, can become lodged there and influence breathing. Techniques for initiating tissue dialogue and releasing pleural spaces congested with exudates are also discussed, highlighting the importance of structural and emotional reorganisation of the lungs to improve respiratory health.

LUNG PRACTICE

The **posterior hand** is slightly lower than the **anterior hand**, which is slightly higher. This is due to the **rocking** movement of the lungs on their axis. This movement is essential for entering the **embryonic field** of the lungs, where the **embryonic fulcrum point** is sought. This point represents a field of aspiration, allowing for repotentialisation in the primary function of the lungs, i.e., being ready to **take the breath of life**.

During the nine months of embryonic development, the lungs are prepared for this first movement. They produce **amniotic fluid**, a crucial function as the embryo needs it for survival. This fluid partly comes from kidney urine. Later, a **specific protein** called **surfactant** will be produced, allowing the lungs to open at birth.

Initially, the pressure applied is gentle. The aim is to gradually find a space where the lung can organise itself and regain its freedom. Between the **visceral pleura** and the **parietal pleura**, there is an interface between the trachea, bronchus, and the aerial bronchial tissue, which contains air and a residual volume, as well as a fluid space.

It is important to note that this tissue can harbour emotions, such as internalised **sadness**. For example, a child who cries in their lungs or suffers from bronchitis may be expressing a sadness they cannot verbalise. By working gently on this tissue, it is possible to initiate a dialogue and **tissue oxidative** exchange.

Through the **great posterior fissure**, one can access spaces where emotions are hidden. It is essential to let the body work and focus on the axis of reorganisation. The **costopleuropulmonary recesses** are sacs for the accumulation of exudates, where lung waste is deposited. These areas, often less mobile, need to be energised and released.

It is also crucial to free the lungs from their attachment points, particularly the **suspensory ligaments of the pleural dome**. The recesses, such as the **costodiaphragmatic recesses** and the **phrenomediastinal recess**, play an important role in the freedom of the mediastinum and heart.

Amniotic fluid production begins at the **fifth week** of pregnancy and develops until the **38th week**, with the appearance of surfactant around the **25th week**. Pulmonary pathologies, such as bronchitis or bronchiolitis, can be manifestations seeking to resolve unexpressed problems during the embryonic stage, appearing as **fluid**. One could say that one "cries in the lungs".

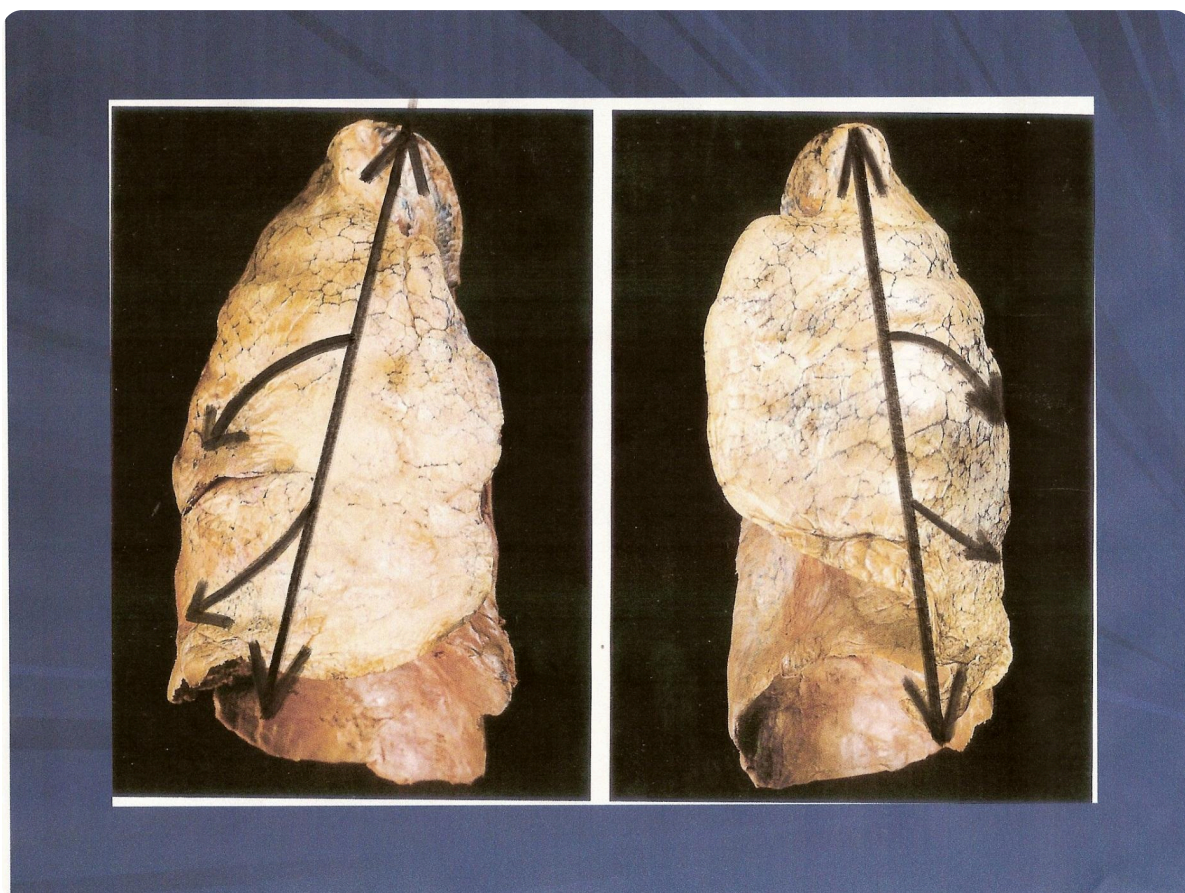


FIG. — Superior oblique lobe

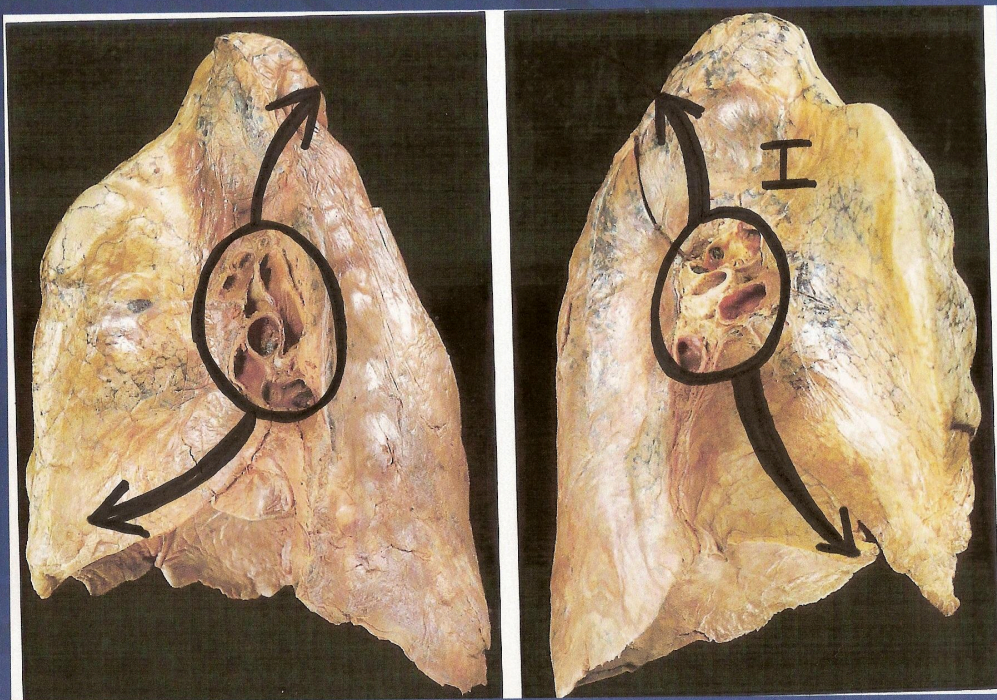


FIG. — Cystic lung disease